

2021년 미국류마티스학회 류마티스관절염 치료 임상진료 guideline

2021 American College of Rheumatology Guideline for the Treatment of Rheumatoid Arthritis

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감수 및 논평: 성윤경, 한양외대 류마티스내과

목적. 류마티스관절염(rheumatoid arthritis, RA)의 약물 치료에 대한 최신 임상진료 지침을 개발한다.

방법. 저자들은 population, intervention, comparator, outcome (PICO)에 따라서 구조화된 핵심 질문을 만들었다. 이에 따라서 체계적인 문헌 검색을 진행하였으며, 근거 확실성의 등급화는 Grading of Recommendations Assessments, Development and Evaluation (GRADE) 방법을 사용하였다. 임상 의사와 환자로 구성된 패널은 투표를 통하여 권고 여부(찬성 또는 반대)와 등급(강한 권고 또는 조건부 권고)에 대한 합의를 진행하였다.

결과. 본 지침은 전통적 합성 항류마티스제, 생물학적제제, 표적

합성 항류마티스제를 포함하는 항류마티스제의 사용과 글루코코르티코이드의 사용, 특정한 고위험군(간질환, 심부전, 림프증식 질환, 과거 심각한 감염 및 비정형미코박테리아 폐질환 환자)에서의 약물 치료가 포함된다. 본 지침은 총 44개의 권고문(강한 권고 7개, 조건부 권고 37개)을 포함하고 있다.

결론. 본 임상진료 지침은 임상 의사와 환자의 의사결정을 돕는 도구로 사용하기 위해 개발되었다. 권고 사항은 규범적인 것이 아니며 개별 환자에 대한 치료 결정은 환자의 가치관, 목표, 선호도 및 동반질환에 기초한 환자와 의사의 공유의사결정을 통해 이루어져야 한다.

COMMENTARY

2015년 ACR 가이드라인으로부터 6년만에 update된 RA 가이드라인으로서, 환자 경험과 선호가 매우 높은 비중으로 반영되었다는 것이 큰 발전이라 할 수 있다. 메토크세이트는 여전히 RA 치료의 가장 중심적 역할을 하고 있는데, 특히 단독 요법을 중심으로 권고하고 있다. 낮은 활성도의 환자에서는 하이드록시클로로퀸을 우선 고려하는 것이 중요한 변화이며, 중등도 이상의 환자에서 메토크세이트에 저항성이 있을 경우, 3제 요법보다는 생물학적제제 혹은 표적합성 치료제를 추가하는 것은 임상 상황과 환자의 선호를 반영한 변화라 할 수 있다. 이외에도 환자가 안정적일 때 순차적으로 어떠한 약제를 감량할 것인지, 초기 치료 결정 시에 스테로이드제를 최대한 지양하는 것 등이 큰 메시지라 할 수 있다. 이와 더불어 심부전, 류마티스결절, 지방간, 폐질환 등을 동반한 환자에 따른 약제 선택에 대해 자세한 가이드라인을 제시하였는데, 이러한 변화는 기존 가이드라인에 의사가 궁금한 내용과 환자의 선호를 충분히 반영하여 활용 가능성을 높였다는 데 의미가 있다.

Table 2. Disease-modifying antirheumatic drugs (DMARDs) initiation*

Recommendations	Certainty of evidence	Based on the evidence report(s) of the following PICO(s)†	Evidence table(s), in Supp. App. 2
Initiation of treatment in DMARD-naïve patients with moderate-to-high disease activity			
Methotrexate monotherapy is strongly recommended over: Hydroxychloroquine or sulfasalazine bDMARD or tsDMARD monotherapy Combination of methotrexate plus a non-TNF inhibitor bDMARD or tsDMARD¶	Very low/low‡ Very low/moderate Low/very low	PICO 2a.C1/C2 PICO 5a.C1–4/C5§ PICO 6a.C2–4/C5§	p. 14–5 p. 61–78 p. 109, 117–28
Methotrexate monotherapy is conditionally recommended over: Leflunomide Dual or triple csDMARD therapy¶ Combination of methotrexate plus a TNF inhibitor¶	Low Moderate Low Very low	PICO 2a.C3 PICO 4a.C1–C2 PICO 6a.C1 PICO 7a	p. 18 p. 46–9 p. 110 p. 167
Initiation of a csDMARD without short-term (<3 months) glucocorticoids is conditionally recommended over initiation of a csDMARD with short-term glucocorticoids.	Very low	PICO 7a	p. 167
Initiation of a csDMARD without longer-term (≥3 months) glucocorticoids is strongly recommended over initiation of a csDMARD with longer-term glucocorticoids.	Moderate	PICO 8a	p. 170
Initiation of treatment in DMARD-naïve patients with low disease activity			
Hydroxychloroquine is conditionally recommended over other csDMARDs.	Very low	PICO 1a.C1–4	p. 1–6
Sulfasalazine is conditionally recommended over methotrexate.	Very low	PICO 1a.C2	p. 2
Methotrexate is conditionally recommended over leflunomide.	Very low	PICO 1a.C3	p. 5
Initiation of treatment in csDMARD-treated, but methotrexate-naïve, patients with moderate-to-high disease activity#			
Methotrexate monotherapy is conditionally recommended over the combination of methotrexate plus a bDMARD or tsDMARD.**	Moderate/very low	PICO 6b.C1–4/C5§	p. 136–56

* PICO = population, intervention, comparator, and outcomes; Supp. App. 2 = Supplementary Appendix 2, available on the *Arthritis & Rheumatology* website at <http://onlinelibrary.wiley.com/doi/10.1002/art.41752/abstract>; bDMARD = biologic DMARD; tsDMARD = targeted synthetic DMARD; TNF = tumor necrosis factor; csDMARD = conventional synthetic DMARD.

† The closest matching PICO questions to each recommendation are provided.

‡ The first certainty of evidence applies to the first listed option; the second certainty of evidence applies to the second listed option.

§ The original PICO included individual DMARDs as comparators. The recommendation considers bDMARDs as a group.

¶ The direction of the beneficial effect is in favor of the nonpreferred option.

Other recommendations for this patient population are the same as those for DMARD-naïve patients.

** The direction of the beneficial effect is in favor of the nonpreferred option. The certainty of evidence is high for the combination of methotrexate plus a TNF inhibitor and moderate for other bDMARDs.

The in-text citation of Appendix C, D, E and F was changed to Appendix 3, 4, 5 and 6 on 08 July 2021 after original online publication.

Table 4. Treatment modification*

Recommendations	Certainty of evidence	Based on the evidence report(s) of the following PICO(s)	Evidence table(s), in Supp. App. 2
A TTT approach is strongly recommended over usual care for patients who have not been previously treated with bDMARDs or tsDMARDs.	Low	PICO 12.a	p. 191
A TTT approach is conditionally recommended over usual care for patients who have had an inadequate response to bDMARDs or tsDMARDs.	Very low	PICO 12.b	p. 199
A minimal initial treatment goal of low disease activity is conditionally recommended over a goal of remission.	Low	PICO 13	p. 201
Addition of a bDMARD or tsDMARD is conditionally recommended over triple therapy for patients taking maximally tolerated doses of methotrexate who are not at target.	Very low	PICO 19.C2–C6†	p. 240–1
Switching to a bDMARD or tsDMARD of a different class is conditionally recommended over switching to a bDMARD or tsDMARD belonging to the same class for patients taking a bDMARD or tsDMARD who are not at target.	Very low	PICO 24–27†	p. 293–338
Addition of/switching to DMARDs is conditionally recommended over continuation of glucocorticoids for patients taking glucocorticoids to remain at target.	Very low	PICO 23	p. 292
Addition of/switching to DMARDs (with or without IA glucocorticoids) is conditionally recommended over the use of IA glucocorticoids alone for patients taking DMARDs who are not at target.	Very low	PICO 28.C1–C2	p. 339–40

* PICO = population, intervention, comparator, and outcomes; Supp. App. 2 = Supplementary Appendix 2, available on the *Arthritis & Rheumatology* website at <http://onlinelibrary.wiley.com/doi/10.1002/art.41752/abstract>; TTT = treat-to-target; bDMARDs = biologic disease-modifying anti-rheumatic drugs; tsDMARDs = targeted synthetic DMARDs; IA = intraarticular.

† The original PICO included individual DMARDs as comparators. The recommendation considers bDMARDs as a group.